

Q I'm eager to deliver the placenta as quickly as possible. Are there any ways to do this?

As with the delivery of your baby, if you stand or sit, gravity will help to push out the placenta and membranes. Be careful if you stand; make sure your birth partner is there to support you, since you will probably feel wobbly after the exertion of giving birth. In addition, holding your baby "skin-to-skin" or starting to breast-feed will increase your body's release of oxytocin, which can speed up the process. Delivering your placenta naturally shouldn't affect your ability to get to know your baby—usually the cord is long enough for you to hold (and even feed) your baby while he is attached.

Q Can we keep the placenta?

If you're having your baby in a hospital or birthing center, you'll need to check the policy on whether or not you'll be allowed to keep your placenta. Even when it's fine to do so, you'll probably have had to seek permission in advance. Hospitals won't have any containers for you to take the placenta away in, so make sure you've packed one in your hospital bag—a plastic container with a secure lid should do.

Q I always thought the umbilical cord was cut immediately, but my doctor said it probably won't be. Why?

When your baby is born, blood flow between him and his placenta continues through the cord for about ten minutes or so after birth. For this reason the clamping of the umbilical cord may be deferred for one to five minutes to allow the extra blood to boost your baby's iron reserves. Adequate iron is important for your baby's early neurological development. Once the cord has stopped pulsating (pushing the remaining blood into your baby's body), your doctor will let you know that it's time to clamp and cut it. She will ask if your birth partner would like to cut the cord, or you may have already requested this in your birth plan. Deferred cord clamping can be carried out for both active and natural third stage labors and C-sections.

Q I'm worried that if my partner doesn't cut the cord properly the baby will have a funny belly button. Is this the case?

The look of your baby's belly button has entirely to do with the way the umbilical cord was attached to the baby in the uterus and has nothing to do with the way it is cut after birth.

If your partner wants to take an active role and cut the cord, encourage him or her to do so. Some dads feel this is a good way to play a significant role in the first moments of their new baby's life, setting the baby free in the world. If your birth partner is eager to do the honors, make a note of it in your birth plan and mention it to your doctor.



Did you know...

Some people eat the placenta. This practice is called placentophagy. While many view the placenta as a waste product of birth, others believe it can benefit the mother, delivering an iron boost and helping to ward off postpartum depression. There are references to eating it in various cultures, and while in the West the placenta has traditionally been incinerated, there is a growing trend in eating it. Evidence on the benefits is anecdotal, but there is no doubt that it is rich in nutrients and hormones.

Q What is the procedure for cutting the cord?

Cutting the umbilical cord is a significant moment when your baby makes the full transition from life in the uterus to an independent existence in the outside world.

Shortly after the birth of your baby, the doctor or your partner will cut the umbilical cord to release your baby from the placenta. First the cord will need to be clamped to prevent bleeding from the baby or the placenta. The cord is clamped in two places. A plastic clip is placed 1½-2 inches (3-4cm) from your baby's belly button, then a second

clip is placed at the other end of the cord near the placenta. The cord is then cut between the two clamps. This procedure isn't at all painful for your baby since there are no nerves in the umbilical cord. Your baby will be left with a stump where the cord was, which will gradually blacken and fall off naturally around 5 to 15 days after the birth.



Cutting the cord This procedure is a straightforward one that can be done by your doctor or birth partner if he or she would like.